

MONROE COUNTY FIRE RESCUE — TRAUMA STAR

PRACTICE CHART — FICTITIOUS PATIENT — FOR QI TRAINING ONLY — NOT A MEDICAL RECORD

Practice Chart 3: IFT, STEMI (target: 100%)

Date / OCA	2026-07-22 / PC-1003	Aircraft	Trauma Star North
Mission	Interfacility (Air)	Chief Complaint	STEMI, chest pain
Pilot	A. Turner	Nurse / Medic	K. Lane, RN / J. Ford, Paramedic
Pickup / Dropoff	Fishermen's Hospital	Kendall Regional Cath Lab	
Dispatch / Wheels Up	1130 / 1137	Land / Depart	1152 / 1208

Flight Narrative

Requesting agency: Fishermen's Hospital ED. Requesting provider and sending physician Dr. Maria Voss, MD; accepting physician Dr. Lee Grant, MD, interventional cardiology, Kendall Regional. Diagnosis: acute ST elevation myocardial infarction, inferior. STEMI is a time-sensitive emergency; door-to-balloon window applies. Sending facility lacks cardiac catheterization capability, not available at the pickup location.

HPI: 58-year-old with crushing chest pain onset 1040, diaphoresis and nausea; events leading to transfer: ECG showing inferior STEMI. Prior to arrival: aspirin 324 mg, heparin bolus given by the sending facility. Infusions running prior to assessment: heparin 1000 units/hr. Labs: troponin 2.8, remaining labs pending; chest x-ray unremarkable per sending facility.

Rotor-wing air transport chosen because ground transport time approximately 88 minutes per Google Maps versus 22 minutes by air; the time savings is clinically significant for myocardial salvage in this STEMI patient. Requires continuous monitoring and titration beyond the scope of available ground providers. Transported to Kendall Regional, whose cath lab capability meets this patient's needs.

1155 BP 128/78 HR 92 RR 18 SpO2 97% on 2L NC. Temp 98.8 F. Cabin temp 72 F. Glucose 156. Pain 8/10; fentanyl 50 mcg IV at 1158; reassessed 1208 pain 5/10; nitroglycerin infusion titrated from 10 to 20 mcg/min at 1210 for continued chest pain, reassessed 1218 pain 3/10, BP 118/72 maintained. 1218 BP 118/72 HR 88 SpO2 98%. 1228 BP 120/74 HR 86 SpO2 98%; vital signs maintained on current infusion rates with serial reassessment.

Max altitude 2000 ft. Radio report given to Kendall Regional at 1220; cath lab team activated by receiving facility. Patient remains at high risk of deterioration without continuous critical care monitoring; care transferred to cath lab team.

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